

Contraception — Counseling & Method Selection

Patient-centered choice over efficacy-first scripts · US MEC/US SPR 2024 · TeachIM (CC BY-NC 4.0), optimized 2026 · clinician-review before use

1 · Open with PATH

- **Pregnancy attitudes:** "Want (more) children at some point?"
- **A/Timing:** "When might that be?"
- **How important:** "How important to prevent pregnancy until then?"
- **Method preference:** "What's most important your birth control does?" / "What have you tried — what worked?"
- Open-ended, no assumptions; inclusive of all patients.

2 · Match method to what matters

Method	Bleeding	Upkeep
Copper IUD	Heavier/crampier	10–12 yr
LNG IUD / implant	Lighter → none; early spotting	3–8 yr
DMPA (Depo)	Lighter → amenorrhea	q3 mo
Pill/patch/ring	Lighter or regular menses	day/wk/mo

- Most effective reversible: **implant** > LNG/copper IUD.
- Bleeding settles by 3–4 cycles; estrogen-free may spot 6–12 mo.
- Prolonged spotting on progestin → oral estrogen if no contraindication.

3 · Choosing an OCP

- **Default:** ~20 mcg ethinyl estradiol + a progestin; keep 2–3 favorites.
- ↓estrogen → ↑breakthrough bleeding; ↑estrogen → ↑thrombosis.
- Progestin gen 3rd/4th = anti-androgenic but ↑VTE — reserve for that need.
- Skip menses: drop placebo pills, start next pack. No "Lo" in name ≈ 35 mcg EE.

4 · Safety — estrogen red-flags

Condition	Avoid
Smoking + age >35 (≥15/day)	Combined (MEC 4)
Migraine with aura	Combined
VTE/CVD/CVA; postop immob.	Combined (4)
Breast cancer	ANY hormonal
Cervical/uterine CA, anatomic, active PID	IUD placement
Lamotrigine	Combined (MEC 3): EE ↑clearance, ↓levels

US MEC: 1 unrestricted · 2 benefit>risk · 3 usually not rec'd · 4 do not use.

5 · Emergency contraception (most → least effective)

- **Copper IUD** — >99%, ≤5 days, BMI-independent (most effective).
- **Ulipristal (Ella)** — best oral, esp. days 3–5 & higher BMI.
- **Levonorgestrel (Plan B)** — ↓efficacy as BMI rises.
- **Yuzpe** — 100 mcg EE + 0.5 mg LNG/dose (~4–5 pills) q12h×2; most available.
- LNG IUD non-inferior to copper for EC (off-label).

7 · Typical-use efficacy & pearls

Method	Typical-use efficacy
Implant	>99.9% (most effective)
LNG / copper IUD	>99%
DMPA (Depo)	~94% (6% fail)
Pill / patch / ring	~93% (7% fail)
pH modulator (Phexxi)	~86%

- Spotting is the #1 reason patients quit early — counsel up front.
- Offer condoms for STI protection with any method (dual protection).
- Negative UPT can't exclude pregnancy if unprotected sex